Hospital Connect

Build Your Own · Option 3 · Team 8 · One-page problem summary

Who has the problem?

A regional health authority or rural ambulance dispatch hub in British Columbia. A lead dispatcher or regional operations supervisor coordinating facilities, crews and resource requests.

What happens today, and why does it matter?

Resource and staffing availability are scattered across calls and updates. A rural team may spend time contacting facilities without knowing which can accept a resource request. This is the team's working hypothesis, not a measured finding. Delay, repeated coordination work, crew workload and trust in availability information. We have not measured time saved, cost savings or clinical outcomes.

One core workflow and four must-haves

Synthetic intake → explained demo priority → find an open resource → request → hospital offer → accept → complete. Must-haves: synthetic intake and explained demo priority; open-facility resource matching; saved request/offer/accept/complete workflow; synthetic-data checks and exports.

Reluctant user and design choice

A dispatcher under pressure may resist another screen or duplicate entry. Use a short intake, explain every flag, show freshness, and retain direct phone confirmation. These design assumptions need an interview.

Data care, assumptions and scope

All identities, health identifiers, inventories, capacity and crew statuses are fictional. Network requests omit identity and medical background. The public demo has no real staff authentication. Screening is illustrative rule logic, not trained AI or clinical triage. Distance is straight-line; 12-hour fatigue markers and review timers are assumptions. Duplicate flags require human review and never deny coordination. Payment approval is a no-charge demonstration. No real alerts, transfers or payments occur.

Platform boundary

Piezas owns hosting and saved records. Our app owns validation, matching, minimisation and workflow explanations. Real use needs authorised staff access, verified capacity, privacy/security review, clinical governance and a payment provider.

Evidence and next validation

No interview yet: confirmed by the team. The team believes rural BC has medical staff shortages. News links below are secondary context from its draft, not verified user evidence. Interview a dispatcher or operations supervisor about a recent difficult transfer; record their role, date, consent and actual observations. Do not claim measured savings or outcomes.

Team-provided secondary context

CBC: BC ambulance / paramedic pressure (9.6985466). CTV Vancouver: fewer paramedics on shift as BC Ambulance millions over budget. Links are in the submission requirement mapping.

Live demo: https://hospital-connect-2179.apps.piezas.ai/workspace.html